

Spinal Arthritis and Back Pain UK: Symptoms, Treatment and S

UK guide to spinal arthritis and back pain. Understand spondylosis symptoms, NHS diagnosis, exercises for spinal OA and when to see your GP about back stiffness.

By **Living With Arthritis Editorial Team** · Reviewed by **Clinical Advisory Panel** (HCPC-registered physiotherapy & rheumatology nursing input) · 2026-01-10 · livingwitharthritis.org.uk



*Person with good posture at an ergonomic desk setup, supporting spinal health
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Spinal arthritis — medically known as spondylosis — is one of the most common causes of chronic back and neck pain in the UK. Affecting the facet joints, discs and

vertebrae of the spine, it's estimated that **over 80% of people over 40** show some evidence of spinal degeneration on X-ray, though not all experience symptoms. This guide explains what spinal arthritis is, how it's diagnosed, and provides practical self-help strategies recommended by UK healthcare professionals.

What Is Spinal Arthritis?

The spine is made up of 33 vertebrae connected by facet joints at the back and intervertebral discs at the front. Spinal arthritis most commonly involves osteoarthritis of the facet joints (facet joint arthropathy) and degeneration of the intervertebral discs (degenerative disc disease). These changes occur naturally with age but can be accelerated by injury, genetics, obesity and occupational factors.

Areas Affected

- Cervical spine (neck) — Causes neck pain, stiffness and sometimes headaches. May cause nerve compression leading to arm pain, tingling or weakness
 - Thoracic spine (mid-back) — Less commonly affected due to the stabilising effect of the rib cage
 - Lumbar spine (lower back) — The most common area for spinal arthritis, causing lower back pain, stiffness and sometimes leg symptoms (sciatica)
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Symptoms



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- Stiffness and pain in the back or neck, worse in the morning and after inactivity
- Pain that improves with gentle movement but worsens with prolonged standing or walking
- Reduced flexibility — difficulty bending, twisting or looking over your shoulder
- Grinding or crunching sensation (crepitus) with movement
- Pain that radiates into the buttocks, hips or legs (lumbar) or shoulders and arms (cervical)
- Numbness, tingling or weakness in limbs (if nerves are compressed — seek medical attention)

Exercises for Spinal Arthritis

Exercise is the cornerstone of spinal arthritis management. These exercises should be performed daily:

Lower Back

- Pelvic tilts — Lie on your back with knees bent. Gently flatten your lower back against the floor by tightening your abdominal muscles. Hold 5 seconds, release. Repeat 15 times
- Knee rolls — Lie on your back with knees bent. Slowly roll both knees to one side, keeping shoulders flat. Hold 10 seconds, then roll to the other side. Repeat 10 times each side
- Cat-cow stretch — On hands and knees, slowly arch your back upward (cat), then dip it downward (cow). Move gently between positions 10 times
- Bridge — Lie on your back, knees bent. Lift your hips off the floor until your body forms a straight line from shoulders to knees. Hold 5 seconds. Repeat 10 times

Neck

- Chin tucks — Sit with good posture. Draw your chin straight back (creating a "double chin"). Hold 5 seconds. This strengthens deep neck flexors. Repeat 10 times
- Gentle neck rotations — Slowly turn your head to look over each shoulder. Hold 5 seconds each side. Repeat 5 times
- Neck side bends — Tilt your ear towards your shoulder (don't raise the shoulder). Hold 10 seconds each side

Core Strengthening

A strong core supports the spine and reduces load on arthritic joints:

- Dead bug — Lie on your back, arms pointing to the ceiling, knees bent at 90°. Slowly extend one arm overhead while extending the opposite leg. Return and switch sides. 2 sets of 10
- Side plank (modified) — Lie on your side, propped on your elbow, knees bent. Lift your hips off the floor. Hold 10–20 seconds. This targets the obliques that stabilise the spine

Posture and Ergonomics



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- Sit with your back supported — use a lumbar roll or rolled towel in the curve of your lower back
- Adjust your computer screen to eye level to avoid sustained neck flexion
- Take movement breaks every 30 minutes — prolonged sitting increases spinal stiffness
- Sleep on a supportive mattress — medium-firm is generally recommended. Place a pillow between your knees when side-sleeping
- Lift objects by bending your knees and keeping the load close to your body

NHS Treatment Options

- Physiotherapy — Tailored exercise programmes, manual therapy and education. Available through GP referral

- Pain medication — Paracetamol, NSAIDs (topical and oral), and sometimes neuropathic pain medications for nerve-related symptoms
 - Facet joint injections — Corticosteroid injections into the affected facet joints, performed by pain specialists or radiologists
 - Pain management programmes — Multidisciplinary programmes combining exercise, psychology and education
 - Surgery — Rarely needed. Options include decompression (removing bone pressing on nerves) or spinal fusion in severe cases
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When to See Your GP Urgently

Most back pain from spinal arthritis is manageable, but see your GP urgently or attend A&E; if you experience loss of bladder or bowel control, numbness in the genital/saddle area, sudden severe weakness in both legs, or worsening numbness/weakness in arms or legs. These symptoms may indicate cauda equina syndrome or cord compression, which require emergency assessment.

Spinal arthritis is extremely common and usually very manageable with regular exercise, good posture habits and appropriate treatment. Our virtual assistant can help you create a tailored back exercise programme.

This article is for general information only. See your GP for persistent back or neck pain, particularly if accompanied by leg or arm symptoms.

Q&A

Question: What Is Spinal Arthritis?

Short answer: The spine is made up of 33 vertebrae connected by facet joints at the back and intervertebral discs at the front. Spinal arthritis most commonly involves osteoarthritis of the facet joints (facet joint arthropathy) and degeneration of the intervertebral discs (degenerative disc disease). These changes occur naturally with age but can be accelerated by injury, genetics, obesity and occupational factors.

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team

promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Versus Arthritis: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.

General information only — not medical advice. Consult your GP or specialist before changing medication or starting a new programme. Emergency: 999. NHS advice: 111. Living With Arthritis UK · livingwitharthritis.org.uk